SACTCHECK · NCCP 00279

My complete treatment handbook

Cisplatin + etoposide with radiotherapy

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

Contents

1. Your three-page treatment companion — page 2
2. Full treatment reference: medicines, schedule, tests, symptoms and follow-up — page 5
3. Detailed treatment and monitoring plan — page 18
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6. Your treatment passport and symptom record — page 23
7. Wallet card and phone instructions — page 28

SACTCheck v0.84.32 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

Cisplatin + etoposide with radiotherapy

Chemotherapy and radiation work together to treat limited-stage small-cell lung cancer. Your team explains your treatment aim and choices.

Limited-stage small-cell lung cancer · combined chemotherapy and radiation · NCCP 00279 v8 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

For limited-stage lung cancer: IV cisplatin/etoposide every 21 days. Oral, split-dose, carboplatin or 28-day plans need personalised information.

My overall treatment

Reference IV plan: four 21-day cycles. Day 1: cisplatin + etoposide. Days 2–3: etoposide. Radiation follows its own appointments. Capsules, split cisplatin, carboplatin substitution or 28-day cycles require a different personalised prescription.

Chemotherapy	Radiotherapy	After treatment
Three treatment days per reference cycle, then recovery. Your team confirms each cycle after safety checks.	Usually starts with cycle 1. Radiation oncology sets the dose and visits; chemotherapy delays need separate coordination.	Your team reviews recovery, persistent symptoms and response. Scan or marker timing is individual.

A typical 21-day cycle

Day 1	Day 2–3	Day 4–7	Day 8–21
reference combination	Days 2–3: etoposide	Support and recovery	Between chemotherapy cycles

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Blood count, kidneys and liver** Required by regimen	Before treatment
Before	**Audiology / creatinine clearance** If clinically indicated	When clinically indicated
During	**Bloods, hearing and nerves** Required by regimen	Before every chemotherapy cycle
After / review	**Recovery and response** Team-agreed plan	Team-agreed timing; no universal interval

Between treatments

- Follow the prescribed anti-sickness course and fluid advice; call early if eating or drinking becomes difficult.
- Fever, new breathlessness, uncontrolled vomiting or inability to drink needs urgent advice, including after treatment ends.

Confirm your prescription, radiation appointments and blood tests. Bring your medicine list and daytime / overnight contact numbers.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hearing / nerves

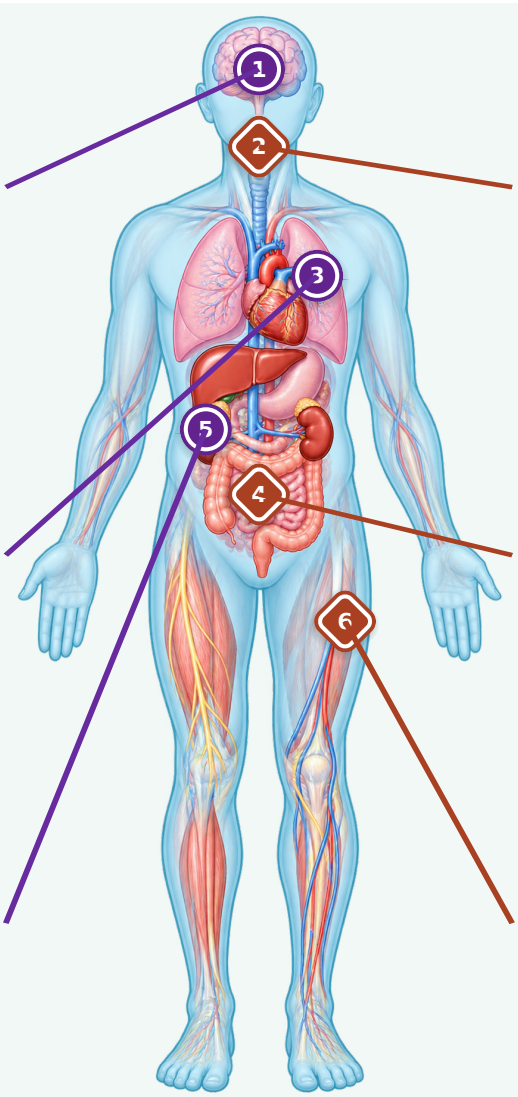
Cisplatin

Report ringing, reduced hearing, numbness or weakness before the next dose.

2 Mouth / swallowing

Treatment and chest radiation

Soreness may affect food and fluids. Contact the team early; inability to drink is urgent.



3 Breathing

Chemoradiation

New or worsening breathlessness needs urgent assessment, including after treatment finishes.

4 Sickness / appetite

Chemotherapy

Follow the anti-sickness plan. Call if vomiting continues or fluids will not stay down.

5 Kidneys / fluid balance

Cisplatin

Blood tests and fluids protect kidneys. Report much less urine or poor intake.

6 Blood cells / IV site

Chemotherapy

Fever or sudden illness can be serious. Report bleeding or infusion pain promptly.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Keep your treatment details to hand

Limited-stage small-cell lung cancer · combined chemotherapy and radiation · NCCP 00279 v8 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Cisplatin + etoposide / RT

Day 1: both drugs. Days 2–3: etoposide. Usually every 21 days ×4; RT plan separate.

Phase: _____ Planned cycles: 4

Daytime (EXAMPLE) 087 12345678
Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1 Date / initials

C2 Date / initials

C3 Date / initials

C4 Date / initials

My treatment information

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00279 · VERSION 8

Your treatment, explained

Limited-stage small-cell lung cancer · combined chemotherapy and radiation · NCCP 00279 v8 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Cisplatin + etoposide with radiotherapy

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc-etoposide

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- My treatment overall
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

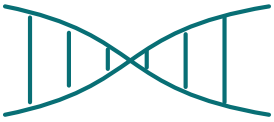
My chemotherapy medicines

Chemotherapy and radiotherapy work together to treat limited-stage small-cell lung cancer. Your team explains the aim and why this combined treatment is suitable for you.

PLATINUM CHEMOTHERAPY

Cisplatin

Platinum chemotherapy



Damages DNA

Cisplatin forms links in DNA that interfere with cell copying. Fluids and kidney checks help reduce harm to healthy tissue.

Treatment reaches cells → Damages DNA → Cancer growth may slow

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What to watch for

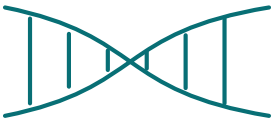
Report poor fluid intake, reduced urine, ringing in the ears or numbness.

Sources: nccp

CHEMOTHERAPY

Etoposide

Chemotherapy



Disrupts DNA repair during cell division

Etoposide blocks an enzyme cells need while copying DNA. The route and prescribed amount differ between infusion and capsules; they are not interchangeable milligram for milligram.

Treatment reaches cells → Disrupts DNA repair during cell division → Cancer growth may slow

What to watch for

Report fever, bleeding, severe mouth soreness or vomiting.

Sources: nccp · smpc-etoposide

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

My treatment overall

DAYS 1–3 EACH CYCLE

Four chemotherapy cycles

1 2 3 4

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

SEPARATE RT APPOINTMENTS

Radiotherapy alongside chemotherapy

Team-agreed

Usually begins with cycle 1; later start may be appropriate. The radiation team confirms dose, number of visits and timing. A chemotherapy delay does not automatically mean radiation stops.

NEXT PLAN AGREED

Recovery and response review

Review

After treatment, your team checks recovery, persistent symptoms and response. Scan type/timing and any useful tumour-marker testing are individual decisions; this regimen does not specify a universal scan or marker timetable.

The chemotherapy course is finite; radiotherapy timing and next treatment are confirmed separately.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Required safety tests are distinct from team-agreed imaging and follow-up; no universal tumour-marker interval is specified.

Check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Chemotherapy	Confirm aim and prescription	Days1–3 each cycle	No additional routine doses	Repeat up to4cycles	Next treatment only if agreed
Radiotherapy	Separate planning appointment	Usually starts cycle1	Follow radiation appointments	RT team confirms ongoing plan	Review late symptoms
Tests	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods before cycle	Follow abnormal results
Symptoms / response	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00279 v8 · official regimen

THE WHOLE TREATMENT JOURNEY

Cisplatin + etoposide with radiotherapy: treatment and checks

Limited-stage small-cell lung cancer · combined chemotherapy and radiation

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Chemotherapy NCCP 00279	Confirm aim and prescription	Days1–3 each cycle	No additional routine doses	Repeat up to4cycles	Next treatment only if agreed
Radiotherapy NCCP 00279	Separate planning appointment	Usually starts cycle1	Follow radiation appointments	RT team confirms ongoing plan	Review late symptoms
Tests NCCP 00279	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods before cycle	Follow abnormal results

Care / check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Symptoms / response NCCP 00279	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

What each check is for

1

Prepare

Before first dose

Chemotherapy and radiotherapy work together to treat limited-stage small-cell lung cancer. Your team explains the aim and why this combined treatment is suitable for you.

Ask / act: Confirm prescription, radiation plan and contacts.

Sources: NCCP 00279

2

Monitor

Each cycle

Check fitness and tolerability

Ask / act: Report symptoms and confirm test results before dosing.

Sources: NCCP 00279

3

Review

After course / when clinically indicated

Assess response and recovery

Ask / act: Confirm the next plan and persistent symptoms.

Sources: NCCP 00279

At the next review, ask

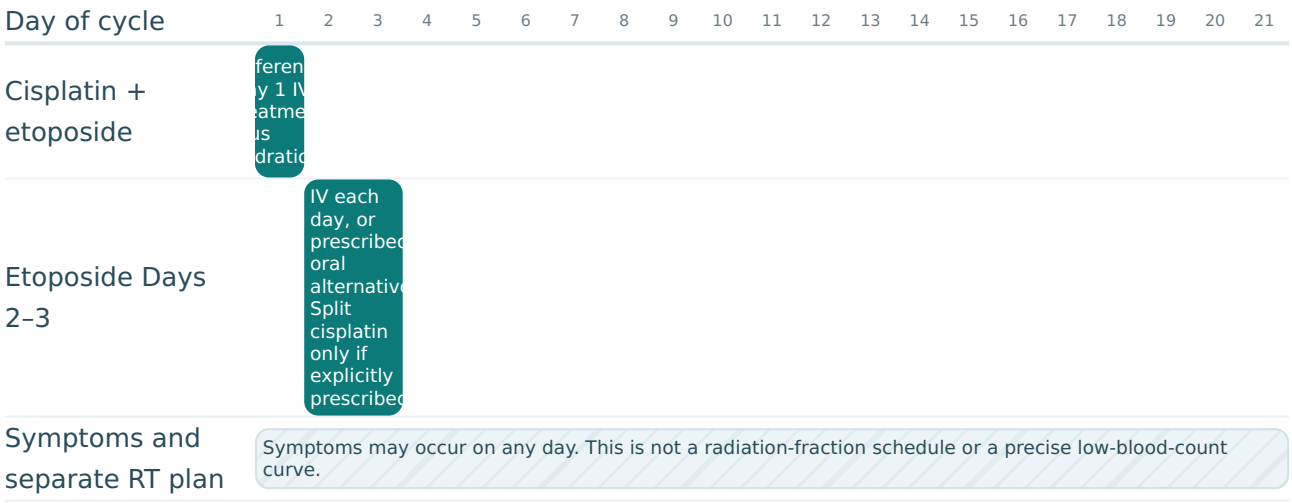
- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.



Relative schedule, not personal dates: Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

No fixed symptom day: Sickness, fatigue, low counts, swallowing problems or breathing symptoms can occur between visits and after treatment. Do not wait for a calendar window to seek advice.

Radiation is a separate schedule: Chemotherapy Day 1/Day 2/Day 3 or weekly counts are not radiation fractions. The radiation oncology team supplies the exact RT timetable.

> **Why these bands stay broad:** Reference cycle without patient dates. RT visits and dose are separately prescribed; the symptom band is not a nadir prediction. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00279 v8 · official regimen ↗

> **There are no guaranteed “safe days”.** Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty,

chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Read the whole cycle as text

Day 1: Day 1: reference combination

Blood results, symptoms, anti-sickness medicines and hydration are checked before both drugs. Tell the nurse about any hearing, nerve or fluid-intake problems.

Day 2–3: Days 2–3: etoposide

IV treatment each day or prescribed capsules twice daily. The split-cisplatin alternative changes these visits. Use your written prescription; do not add IV and oral etoposide together.

Day 4–7: Support and recovery

No routine chemotherapy in the reference plan. Follow ongoing anti-sickness medicines and separate RT appointments. Call early for poor intake, vomiting, swallowing pain or fever.

Day 8–21: Between chemotherapy cycles

Fatigue, mouth symptoms and infection risk can persist. RT may continue according to its own plan. Report symptoms when they occur; do not wait for the next blood test.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Confirm your plan

Reference IV plan: etoposide first on Days 1–3, cisplatin after etoposide on Day 1, each infusion about one hour plus checks and fluids. If prescribed, Days 2–3 etoposide may instead be capsules twice daily. Confirm route, platinum, interval and hydration before treatment; never combine alternatives yourself.



STEP 2

Keep the contact card

The team supplies the daytime and out-of-hours numbers. Keep the card with you and show it when seeking medical care.



STEP 3

Two linked treatment teams

Confirm which team to contact for radiation or chemotherapy changes. Report swallowing problems early and ask about transport, nutrition and fatigue support.

Open practical treatment guide · **Printable practical guide**

Sources: NCCP 00279 v8 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Fluids and sickness prevention

Cisplatin needs a planned fluid and anti-sickness course. Follow the labelled medicines for each treatment day and the days afterwards. Call if vomiting prevents drinking or taking medicines.

2 If capsules are prescribed

Etoposide capsules on Days 2–3 replace the IV doses on those days. Take the prescribed amount twice daily on an empty stomach. Follow pharmacy handling instructions and contact the team if a dose is missed or vomited; do not take an extra dose without advice.

3 Radiotherapy support

Ask about swallowing, eating, skin care and fatigue during radiation. Painful swallowing or poor intake needs early help; inability to drink needs urgent assessment.

Sources: nccp · ncis

Why chemotherapy and radiotherapy are combined

Studies support combining systemic treatment with radiation in the relevant lung-cancer setting.

The evidence comes from specific treatment schedules and patient groups. Your team explains how it applies to your situation; study averages cannot predict an individual outcome.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Which drugs and routes are prescribed for me?

Confirm the exact drug names, routes and interval against your written prescription. The team must explain any change from this reference plan.

How do my radiation appointments fit around chemotherapy?

Radiation has its own calendar. Ask for the combined plan and whom to contact if a chemotherapy or radiation visit changes.

Which blood tests are required before the next dose?

Use the checks table as a prompt. Your team confirms which tests are needed, where they are done and who reviews the results before treatment.

Who do I call during the day and overnight?

Ask the team to replace the example numbers on your card with the correct daytime, overnight and backup contact routes before you leave.

When will recovery and response be reviewed?

Agree the follow-up appointments and any scans with your team. This regimen does not set a universal scan or tumour-marker timetable.

> **Try explaining the plan back:** Explain back your chemotherapy days, how radiation is scheduled separately, the tests before treatment and whom you will call if unwell.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00279 v8. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00279 v8 · official regimen
- NCCP NCIS medical-oncology antiemetics v8
- Etoposide capsules manufacturer product information
- Turrisi et al.1999 · limited-stage SCLC radiotherapy comparison

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://assets.hse.ie/media/documents/279_CISplatin_and_Etoposide_and_RT.pdf

https://assets.hse.ie/media/documents/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://www.medicines.org.uk/emc/product/10533/smpc

https://www.nejm.org/doi/full/10.1056/NEJM199901283400403

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Required safety tests are distinct from team-agreed imaging and follow-up; no universal tumour-marker interval is specified.

Check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
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Tests	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods before cycle	Follow abnormal results
Symptoms / response	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00279 v8 · official regimen

THE WHOLE TREATMENT JOURNEY

Cisplatin + etoposide with radiotherapy: treatment and checks

Limited-stage small-cell lung cancer · combined chemotherapy and radiation

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
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Tests NCCP 00279	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods before cycle	Follow abnormal results
Symptoms / response NCCP 00279	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

What each check is for

1 Prepare

Before first dose

Chemotherapy and radiotherapy work together to treat limited-stage small-cell lung cancer. Your team explains the aim and why this combined treatment is suitable for you.

Ask / act: Confirm prescription, radiation plan and contacts.

Sources: NCCP 00279

2 Monitor

Each cycle

Check fitness and tolerability

Ask / act: Report symptoms and confirm test results before dosing.

Sources: NCCP 00279

3 Review

After course / when clinically indicated

Assess response and recovery

Ask / act: Confirm the next plan and persistent symptoms.

Sources: NCCP 00279

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.30

NCCP 00279 · clinician draft

Cisplatin + etoposide with radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Cisplatin Day 1 · high emetic risk** — High
  Cisplatin + Etoposide
- **Days 2–3 · match actual drugs and route** — Route / schedule dependent
  Etoposide
- **Only when carboplatinAUC5 substitution is prescribed** — High carboplatin AUC≥4
  Carboplatin + Etoposide

https://sactcheck.com/patient/00279/supportive-care.html

Cisplatin + etoposide with radiotherapy

NCCP 00279 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Confirm your plan

Reference IV plan: etoposide first on Days 1–3, cisplatin after etoposide on Day 1, each infusion about one hour plus checks and fluids. If prescribed, Days 2–3 etoposide may instead be capsules twice daily. Confirm route, platinum, interval and hydration before treatment; never combine alternatives yourself.

STEP 2

Keep the contact card

The team supplies the daytime and out-of-hours numbers. Keep the card with you and show it when seeking medical care.

STEP 3

Two linked treatment teams

Confirm which team to contact for radiation or chemotherapy changes. Report swallowing problems early and ask about transport, nutrition and fatigue support.

Sources: NCCP 00279 v8 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

Cisplatin + etoposide with radiotherapy

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital’s instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00279/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

NCCP 00279 V8 · ONE CYCLE, ONE PAGE

Cisplatin + etoposide with radiotherapy

Cycle number: _____ Start date: _____ Next review: _____

Bring your contact card and medication list. Record each chemotherapy treatment and separately confirm the radiation appointments.

Cycle 1 · reference Days 1–3; verify your alternative · copy this sheet for another cycle of the same phase.

Cisplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Urinary symptoms	☐ Usual pattern	☐ Mild change: report	☐ Pain or frequent urination: call	☐ Cannot pass urine / marked bleeding: urgent review	☐ Collapse or severe illness: 112/999
Hearing changes	☐ No change	☐ New ringing or muffled hearing: report promptly	☐ Conversation difficult: contact the team	☐ Sudden hearing loss: urgent assessment	☐ Sudden weakness or collapse: 112/999
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help

Etoposide · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cisplatin + etoposide with radiotherapy

Cycle number: _____ Start date: _____ Next review: _____

Bring your contact card and medication list. Record each chemotherapy treatment and separately confirm the radiation appointments.

Cycle 2 · reference Days 1–3; verify your alternative · copy this sheet for another cycle of the same phase.

Cisplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Urinary symptoms	☐ Usual pattern	☐ Mild change: report	☐ Pain or frequent urination: call	☐ Cannot pass urine / marked bleeding: urgent review	☐ Collapse or severe illness: 112/999
Hearing changes	☐ No change	☐ New ringing or muffled hearing: report promptly	☐ Conversation difficult: contact the team	☐ Sudden hearing loss: urgent assessment	☐ Sudden weakness or collapse: 112/999
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help

Etoposide · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cisplatin + etoposide with radiotherapy

Cycle number: _____ Start date: _____ Next review: _____

Bring your contact card and medication list. Record each chemotherapy treatment and separately confirm the radiation appointments.

Cycle 3 · reference Days 1–3; verify your alternative · copy this sheet for another cycle of the same phase.

Cisplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Urinary symptoms	☐ Usual pattern	☐ Mild change: report	☐ Pain or frequent urination: call	☐ Cannot pass urine / marked bleeding: urgent review	☐ Collapse or severe illness: 112/999
Hearing changes	☐ No change	☐ New ringing or muffled hearing: report promptly	☐ Conversation difficult: contact the team	☐ Sudden hearing loss: urgent assessment	☐ Sudden weakness or collapse: 112/999
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help

Etoposide · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cisplatin + etoposide with radiotherapy

Cycle number: _____ Start date: _____ Next review: _____

Bring your contact card and medication list. Record each chemotherapy treatment and separately confirm the radiation appointments.

Cycle 4 · reference Days 1–3; verify your alternative · copy this sheet for another cycle of the same phase.

Cisplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Urinary symptoms	☐ Usual pattern	☐ Mild change: report	☐ Pain or frequent urination: call	☐ Cannot pass urine / marked bleeding: urgent review	☐ Collapse or severe illness: 112/999
Hearing changes	☐ No change	☐ New ringing or muffled hearing: report promptly	☐ Conversation difficult: contact the team	☐ Sudden hearing loss: urgent assessment	☐ Sudden weakness or collapse: 112/999
Numbness / tingling	☐ None	☐ Mild change; usual tasks unaffected	☐ Limits buttons, writing or walking: call	☐ Washing, dressing or walking difficult: urgent review	☐ Sudden major weakness / paralysis: emergency help

Etoposide · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Limited-stage small-cell lung cancer · combined chemotherapy and radiation · NCCP 00279 v8 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Cisplatin + etoposide / RT

Day 1: both drugs. Days 2–3: etoposide. Usually every 21 days ×4; RT plan separate.

Phase: _____ Planned cycles: 4

Daytime (EXAMPLE) 087 12345678
Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1 Date / initials

C2 Date / initials

C3 Date / initials

C4 Date / initials

My treatment information

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00279 v8 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.